Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION

So you're thinking about pharmacy

The short version — written for you, not about you.

Everything that matters, none of the bits written for your parents.

We scored twenty-seven careers on how exposed they are to AI.

Clinical and hospital pharmacy: **4.0 out of 10.** Community and retail pharmacy: **4.9.**

Those are fine. They're also the weakest scores of any licensed healthcare profession we measured — worse than nursing (2.8), dentistry (2.7), physical therapy (2.5) and veterinary medicine (2.7).

That's surprising for a doctorate-level clinical career, and the reason is genuinely worth understanding.

The 60-second version

- **A licence is only as protective as the act it reserves** — and pharmacy's reserves dispensing.
- Dispensing is a rules-based, structured task. That's what machines are best at.
- But the profession is **moving toward the clinical end**, and clinical pharmacy is growing at ~12%.
- And the job market has completely flipped in the last few years. Most careers advice hasn't caught up.
- Which means: aim at the clinical end deliberately, from the start.

Why the licence doesn't protect as much as you'd think

This is the useful idea, and it applies well beyond pharmacy.

A licence protects a specific *act*. So the question isn't "does this career have a licence" — it's "what does the licence actually reserve, and can a machine do that thing?"

Nursing's licence reserves clinical practice — physical assessment, hands-on care. Those acts are physically irreducible. The licence protects work that couldn't be automated anyway, so the protection compounds.

Law's bar card reserves the courtroom and the signature — but *not* document review. So document review went, and junior law now scores 7.9.

Pharmacy's licence reserves dispensing and verification — which is precisely the structured, rules-based, high-volume task a machine handles well.

So the protection and the exposure land on the same activity. The law requires a human to supervise a process that increasingly doesn't need supervising.

That's why a doctorate-level clinical profession scores in the same band as accounting rather than alongside nursing.

The scores

TRACK	SCORE
Clinical / hospital pharmacy	4.0
Ambulatory & specialty pharmacy	4.4
Community / retail pharmacy	4.9
Pharmacy technician	5.7
Central fill / mail order	6.9

Notice what central fill tells you: a pharmacist verifying a prescription doesn't need to be in the same building. That's why remote and mail-order pharmacy exists at all — and it's the protection pharmacy doesn't have that nursing, dentistry and physiotherapy do.

Now the good news, because it's substantial

The job market has reversed, and most careers advice hasn't noticed.

What used to be true:

- 2000: 78 pharmacy schools, ~7,000 graduates a year
- 2022: 142 schools, **13,323 graduates** — nearly double
- Projected oversupply of 19,000–51,000 pharmacists by 2030
- Debt rising while salaries stagnated

That's the reputation pharmacy still has: expensive degree, saturated market, retail burnout.

What's happened since:

- PharmD applications fell **over 60% between 2011 and 2021** — from 106,000+ to under 41,000
- In 2018, enrolled students **outnumbered applicants** for the first time
- Graduates fell **24% between 2018 and 2024**

And now the arithmetic has flipped: BLS expects **over 14,000 pharmacist openings a year**, while only about **8,000 PharmD graduates** are expected in 2026 — under 60% of what's needed.

One caution. With applications down, some schools have relaxed entry requirements to fill seats. Getting in doesn't mean the market needs you — check a programme's graduate outcomes rather than assuming.

Where the profession is actually going

The direction matters more than the current score.

Pharmacy is deliberately moving *away* from dispensing and *toward* clinical work — medication therapy management, chronic disease care, immunisation, and prescribing authority where it exists.

Clinical pharmacy is projected to grow around 12% — roughly double the rate for pharmacists overall.

Every step toward the patient improves your position, because it swaps a task the licence protects-but-machines-can-do for one that genuinely needs a person.

What actually holds up

Catching what the system missed. Automated checks find rule-based errors. The errors that actually harm people are contextual — the right drug for the wrong patient situation — and that needs clinical knowledge no system has.

Complex regimens. The patient on fourteen medications with kidney impairment. That's real clinical reasoning.

Counselling someone who's frightened about a new diagnosis or a medication they've just read about. Consistently the most valued thing pharmacists do.

And being accountable when something goes wrong.

What we won't soften

The debt is substantial. In 2023, 82.2% of pharmacy graduates had borrowed, averaging around **$168,000** — up from about $134,000 in 2013. Costs have risen faster than inflation since 2004.

Retail is where much of the employment still is, and workforce research has found higher burnout and lower fulfilment among pharmacists at large chains — driven by volume targets, understaffing and expanding duties.

Technicians are absorbing dispensing tasks, which is efficient and also narrows what your licence practically covers.

Does this actually sound like you?

The honest question first: are you interested in *medicines*, or in healthcare generally?

Pharmacy is deep specialist knowledge about drugs. If what you actually want is hands-on clinical work with patients, nursing and allied health offer far more of it, at better scores, for less money.

Pharmacy suits people who:

- Are **genuinely interested in medicines** as a subject
- Are **precise**, and comfortable being responsible where errors harm people
- Want **clinical work without ten to fifteen years of training**
- Are **approachable** — being the accessible healthcare professional is much of the value
- Want **regular hours** in a clinical field

Harder if you want physical hands-on work, need variety (community dispensing is repetitive by design), or dislike retail environments.

The strategy, if you do this

It's clearer here than in most careers:

1. Aim at clinical and ambulatory practice from the start. Don't assume the PharmD gets you there.

2. Plan for residency early. It increasingly separates clinical roles from dispensing ones, and it's competitive.

3. Take prescribing authority wherever it exists. It converts a dispensing role into a clinical one.

4. Specialise — oncology, critical care, infectious disease. That's the protected end and it's growing fastest.

What actually matters when picking a course

- **Residency match rate.** The number that increasingly determines whether you reach clinical practice.
- **How much hospital and ambulatory rotation**, not just community placement?
- **Total cost**, against realistic salaries — the borrowing is significant
- **Where are graduates at three years** — hospital, ambulatory, community, industry?
- **Have entry requirements changed recently?** Fair question given the application decline.

Two things worth doing

Shadow in both a hospital pharmacy and a busy community one. The gap between those two days is essentially this whole document, and it'll tell you which version of this career you want.

Then find a pharmacist three to five years qualified and ask what proportion of their time is actually dispensing versus clinical. The answer varies enormously and it's the thing that matters.

One last thing

Pharmacy gets a worse score than other healthcare careers for a specific structural reason, not because it's a bad choice.

What it offers is genuine clinical work, very high public trust, and regular hours — four years after a bachelor's rather than ten to fifteen. That combination is rare.

Just aim at the right end of it deliberately, rather than assuming the degree takes you there.

If you want to go further — three things worth twenty minutes

- **Pharmacists — Occupational Outlook Handbook** — government data on openings and pay. Where the 14,000-openings figure comes from.
- **AACP institutional research** — the application and enrolment data behind the pipeline reversal. Worth seeing the actual numbers.
- **Why pharmacy applications declined** — a university research centre's analysis, including the debt figures. Academic rather than promotional.

There's a longer version behind this — full scores by track, how they were worked out, the licensing argument in detail, what to ask a program, and where we might be wrong. Your parents have it.